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/ **Practice Exam - 1**

Correct Answer

Next Q

Practice Exam - 1

Question 115 of 117



The patient is a 62-year-old female with chronic stage C heart failure with reduced ejection fraction secondary to nonischemic cardiomyopathy, diabetes, hypertension and CKD. She established care with a cardiologist and her GDMT was adjusted. She is maintained on Entresto 97-103 mg twice daily, carvedilol 6.25 mg twice daily, spironolactone 25 mg once daily empagliflozin 10 mg daily, furosemide 20 mg daily and metformin.

She has remained stable on this regimen for the prior year and now presents to the office complaining of worsening dyspnea on exertion (previously able to walk a mile and now develops dyspnea with 2 blocks), orthopnea and abdominal bloating. On examination, her blood pressure is 115/72 mmHg (stable from prior), her heart rate is 82 bpm and she is saturating 98% on room air. Her JVP appears elevated at 10 cm of h20 with a positive HJR. She has no signs of LE edema or ascites. Blood work is notable for a Creatinine of 1.6 mg/dl and a potassium of 4.7 mEq/L.

Which would be the appropriate next management strategy?

A. Add metolazone 5 mg daily

 B. Add acetazolamide 500 mg daily

C. Increase Spironolactone to 50 mg daily

 D. Increase furosemide to 40mg once to twice daily

Commentary References

Content Code of Blueprint: Acute on chronic heart failure

Task: Treatment

Teaching Point: Exam takers should recognize that patients not responding to low dose loop diuretics should be treated with

escalation of loop diuretic dose prior to addition of thiazide diuretics

Correct Answer:

D. Increase furosemide to 40mg once to twice daily

Rationale:

The patient is presenting with fluid retention and decompensation of her chronic heart failure and needs decongestion. While adding a thiazide diuretic (choice A) does increase diuresis by inhibiting compensatory distal tubular sodium absorption, it increases the risk of electrolyte abnormalities namely hypokalemia and hyponatremia as well as worsening renal function.

Retrospective propensity matched analyses have also suggested increased mortality with the addition of thiazide diuretics.

This approach is supported in the 2022 ACC AHA HF guidelines which state that "addition of thiazide diuretics to loop diuretic should be reserved for patients who do not respond to moderate or high-dose loop diuretics to minimize electrolyte abnormalities". Furosemide 20 mg is a low-dose and should thus be titrated for decongestion and this patient (choice D) .

While RCT data guiding diuretic choices is limited, the DOSE (Diuretic Optimization Strategies Evaluation) trial supports the safety of high-dose diuretics.

In regards to choice B and C, both of these choices are incorrect as neither would result in decongestion which is the primary first goal for this patient. While sequential nephron blockade with adjunctive non loop diuretic is recommended such as adding or increase spironolactone, the first step should be to increase loop diuretics to at least moderate doses.

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